

Dear reader,

Thank you for purchasing a MedLighten booklet, tuition session, course or mock exam! I hope your preparation is going well!

This is a first draft of the communications skills handbook so still in its early stages. As you probably appreciate, time is always tight but I wanted to keep you all updated and give you some extra learning materials.

There's a lot more information being worked on for the final draft and you'll all be sent a copy once it's completed!

I'd love to hear any of your thoughts or opinions on this booklet so please get in touch with any opinions you might have.

Thanks again!

Dr Beej Shah

CSA Revision:

Communication Skills



General Tips

This section contains some general tips on how to adapt your speaking to tick those communication skills boxes.

Interpersonal Skills, Communication Skills and Rapport

Getting a pass on your interpersonal skills, communication skills or getting a good rapport isn't necessarily about using the perfect wording. It's more about how the consultation feels and how you come across.

These tips are intended to give strategies and phrasings that will help the consultation feel and flow better, making sure you leave an excellent impression on the examiner and actor!

First Impressions!

The first words a patient hears when they enter your room and how they're spoken is important! You may be 8 cases into the exam and want to hide in a cupboard, but that first impression always counts! If you're sounding fed up, annoyed or upset then the patient could react to it and you can almost guarantee your ruthless examiner will!

First Responder

Maybe the patient's been sat in your chair for a minute and poured their heart out to you or maybe they've just complained about something small.

Your first response matters!

A good strategy is to reference what the patient says. This shows that you listened to what the patient said, and it'll make you stand out to the examiner too.

Patient:

“Hi doctor, I’ve had a headache for a week now and this paracetamol isn’t helping!”

e.g. 1

“Certainly, we’ll look into that”

This is a generic response that MANY people will use. There’s a good chance the examiner has heard it ALL DAY and is sick of hearing it now. It may be positive, but it doesn’t show that you’ve actually *listened* to the patient and comes across as formulaic.

e.g. 2

“We’ll certainly look into what’s going on with your headaches and see what we can do for them.”

This is positive and shows the patient (and examiner) that you’ve listened to them and will address their concerns.

Some additional tips:

- Don't make promises you can't keep! Agreeing to a "scan" at the start of the consult is only going to create problems for you later when you have to deny them it.
- Make it short! It can be tempting to try and address everything they've just said but this will end up costing you time. The important thing is that they know you've listened!
- Follow up quickly with some questions!

Patient Opener Sample Dialogues:

Below are some openers a patient might use. Try getting someone to use them and then come up with a quick response. There are some example response on the pages which follow. Additionally, you may want to ask a friend to come up with some and see how quickly you can respond.

“Hi doctor, I’ve been having a really rough time lately and I think I need my medication adjusted”

“Hey doc, long story. I got drunk last night and we forgot to use a condom so now I need the morning after pill.”

“Hi, my throat hurts and I think I might need some antibiotics”

“Morning doc, my girlfriend left me and then I went clubbing and now my pee burns and I just don’t know what to do about anything!”

“Finally doctor! I’ve been waiting for 15 minutes now! My eye is really hurting and I need some drops to fix it!”

“Hey doctor, I’m in a massive rush so if we could get this over quickly I’d appreciate it. I just need some pills for this chest infection.”

Example Responses:

“Hi doctor, I’ve been having a really rough time lately and I think I need my medication adjusted”

“I’m sorry to hear that Mrs Smith. Shall we have a chat about what’s going on and then maybe see what we can do about your meds?”

“Hey doc, long story. I got drunk last night and we forgot to use a condom so now I need the morning after pill.”

“Ok, well I should definitely be able to offer you the morning after pill. Could I ask you a few questions just to make sure we get you the right one?”

“Hi, my throat hurts and I need some antibiotics”

“I’ll see what we can do about getting you some antibiotics but could I find out a bit more about your throat first?”

“Morning doc, my girlfriend left me and then I went clubbing and now my pee burns and I just don’t know what to do about anything!”

“Oh, it sounds like you have quite a lot going on there! Would you like to tell me a bit more about what’s been happening?”

“Finally doctor! I’ve been waiting for 15 minutes now! My eye is really hurting and I think I need some drops to fix it!”

“I’m sorry about the wait Mr Jones. I’ll certainly do what I can to help with your eye!”

“Hey doctor, I’m in a massive rush so if we could get this over quickly I’d appreciate it. I just need some pills for this chest infection.”

“Ok, we can definitely take a look at your chest but may I just ask is there a reason you’re in a rush?”

Acknowledging Patients

Whether it's their toe pain that's been troubling them or their back has left them in a wheelchair, it's important that when they tell you these things that you acknowledge them! Not only does it show that you're listening, but your rapport will improve.

The following guidance should help you get the right acknowledgment every time!

1. Don't rush it!

Many people know they need to acknowledge things and do. But if you fire off your acknowledgement too quickly (because you're on a time limit and you've got a list of 100 other questions you need to ask) it sounds incredibly insincere and you're going to shatter your rapport to pieces! Slow your speaking down, say it properly and find other ways to save time.

2. Acknowledge the Good

When you're constantly hearing about what's going wrong in patients' lives and bodies, it's easy to forget to celebrate the good. Remembering to congratulate the patient when they've achieved something or done something good is a great way to build rapport that's so often missed.

Some things to look out for:

- Quitting smoking
- Going sober
- Getting married
- Getting a job
- Meeting a new partner

3. The Quiet Acknowledgement

The acknowledgement you use with the patient whose dad died yesterday is going to be very different from the acknowledgement you use with the patient whose dad died 2 years ago or 10 years ago.

If an event has happened in the patient's past which is significant enough to deserve an acknowledgement (death of a loved one, quitting smoking etc) then a quietly spoken (so as not to interrupt) "I'm sorry" or "congrats" shows that you're listening to what they're saying and being responding appropriately will gain rapport.

4. *“I’m sorry to hear about that...”*

This is a stock phrase that gets used all the time! It’s too general, sounds formulaic and examiners will probably have heard it repeatedly all day. Whilst it *can* be appropriate in some situations, it’s often used inappropriately and will more than likely cost you rapport or damage your impression.

5. Acknowledging Minor Ailments:

A sore toe may be at the bottom of your list of concerns but right now it’s at the top of the patient’s so it should still be acknowledged. The stock phrase “I’m sorry to hear about that” is most likely going to be overkill in this scenario and cost you rapport.

If they've got a small ailment, there aren't going to be any phrases you can fall back on so this is where your Hollywood acting skills come in!

You've probably developed your own reaction expressions to small things as we all talk about small annoyances every day (take, for example, the British tradition of talking about the weather!) so look at other people's reactions, look at your own and find out what works best for you. Good tone and facial expression can get you a long way in this exam so make sure you practice and it comes naturally!

Difficult topics

Scenario: *A patient appears very upset and has opened up to you for the first time about their abusive partner.*

For difficult topics like this, a good choice of words would be “thank you for sharing that with me” followed by a phrase which demonstrates some empathy or that you’ve listened. Your tone should be solemn (showing that you appreciate the seriousness of the situation), your pace should be slow to put emphasis on what you’re saying and you should leave a pause after saying it so that the patient doesn’t feel like they’ve been cut off.

Once you’ve made your acknowledgement, the patient will likely still feel vulnerable so you’ll want to acknowledge this too before you ask any further questions:

“I imagine this may be difficult, but could I just ask you some questions about _____?”

Putting this all together:

Patient:

“My wife has been beating me for years. It started off as little things, but it just got worse and worse and worse. I’m afraid to go home. I hate living with her. I want to leave but I don’t know how!”

Doctor:

“Thank you for sharing that with me. It sounds like you’re in a really difficult situation there and I will do all I can to help you. I imagine this may be difficult, but could I just ask you some questions about the beatings?”

Other topics that you should take a similar approach to include:

- Rape / Sexual abuse
- Bereavement / Death
- Domestic Violence
- Recent diagnosis of a life changing illness

Cues

It is always important to ACKNOWLEDGE cues.

After you ACKNOWLEDGE the cue, you have one of three possible options:

- Follow the cue
- Complete what you are doing
- Ask the patient what they want to start with

The following are general rules for cues. They do not apply in every scenario.

When should you FOLLOW a cue?

- When the cue is a red flag

e.g. 1

Susan (56) has come in wanting a sick note for work related stress. She then tells you she has had a hoarse voice for 9 weeks

“I know you are here for the sick note and we will definitely discuss the issues at work because it sounds like they are really affecting you. However, is it ok if I ask a bit more about your voice first? Because it has been an issue for 9 weeks it is important that we check there is nothing more serious going on.”

e.g. 2

Jan (39) presents with vitiligo and wants a referral to dermatology. You notice she is low in mood and she reveals she is suicidal.

“I know you came here to discuss your skin and we will see what we can do to help this, but as well as helping you with your skin, it is important we keep you safe. I am sorry you are having suicidal thoughts – have you told anyone about this? Could I ask – sometimes when people have thoughts of suicide, they make plans as to how they might end their life – have you made any plans like this?”

When should you **ACKNOWLEDGE** the cue but complete the task in hand?

- When the task in hand is excluding red flags but the cue is not a red flag.
- When you are taking the history of presenting complaint and the patient mentions an unrelated issue that is not a red flag

e.g. 1

Sophie, 28 has come to see you because she is having back pain. You have started taking a back pain history. She then mentions that she is stressed about her accountancy exams and would like a letter of mitigation.

“It sounds like things are quite stressful at the moment, we will certainly discuss the letter you would like, but could I ask a few more questions about your back pain first, just to ensure there is nothing serious going on?”

When should you ACKNOWLEDGE the cue and ask what the patient wants to discuss first?

- When the patient mentions two physical symptoms of equal importance

e.g. Steven, 29 presents with acne. He then mentions dysuria.

Mirroring

When people interact positively, it's human nature for them to copy (mirror) each other's body language and speech patterns.

You can use this to your advantage!

When faced with an angry patient, by slowing down and softening your speech with an open posture you can help them calm down as they will naturally start to mirror you. But...

It can also be your downfall!

An anxious patient will often present as tense with an upright posture talking loudly and rapidly. If you start mirroring this patient, there is a good chance they may become even more anxious.

Side note:

When people in a conversation don't like each other, their postures, tone of voice and the way they speak will be opposites. Sometimes, this can be a good clue that patients are upset with you or something you've said or done.

Data **Gathering**

This section is dedicated to skills concerned mostly with data gathering.

Asking Questions

There are literally *hundreds* of books on how to ask questions! With that in mind, the following practical tips should help your consultation run a lot better

1. Use question words and include “you” or “your” at the start of each question

Most people love to be the centre of attention! Adding words like “you” and “your” to your questions will make the patient feel like you're talking to them and boost your rapport.

e.g. 1

“any vomiting?”

This is short, to the point and would save you time but makes you come across as impatient and impersonal because of this.

e.g. 2

“Have you had any vomiting?”

This is still quite impersonal but those three words makes the patient feel like you are talking to them.

2. Be relevant and specific

If you asked someone with acne “how are your periods” they might tell you they are very painful, they last 5 - 6 days, that they get mood swings and bloating before they start.

What you want to know is whether the patient has oligomenorrhea because you might be considering a diagnosis of PCOS.

e.g.

“How often do you get a period?”

This shows the examiner you know your medicine. It also saves you a lot of time.

3. Short Questions

On their own, short questions *can* make you appear blunt, impatient and impolite so adding a word or two to shorter sentences can help you appear more friendly and personal. Aiming for 5 words and over is a good strategy.

e.g. 1

“Did you drive here?”

This is addressing the patient, specific and relevant but is short and comes across impersonally.

e.g.2

“Did you drive here today?”

This is just one word longer and more specific but comes across a lot more personal.

e.g.3

“Did you drive here today Mr Rogers?”

makes the same question even more personal and creates a better rapport.

The exception to this is when you're trying to speed up the consultation using a series of quick questions.

4. Saving Time with Quick Questioning

Pace

The slower you ask the question, the longer the response will be.

This goes back to the mirroring principle. It's not *a/ways* the case but if you ask a question slowly, the patient will take more time to answer it because they're mirroring you or they think it's significant. Alternatively, they may just add detail to make their response longer.

e.g. 1

Dr: "Do you smoke?"

Pt: "No, I think it's a really bad habit. I tried it once and almost vomited" (long response)

Patient: "No" (quick response)

e.g. 2

Dr: "Are you having any problems driving?"

Patient: "No more than I normally do" (long response)

Patient: "No" (quick response)

You may only save a few seconds, but in 10 minutes this can really add up.

Side note: Some candidates have the opposite difficulty that they speak too quickly which will negatively affect rapport and make the consultation look rushed. The best way to assess your pace is to ask for feedback from colleagues or watch yourself on video.

Signposting

Signposting is telling the patient why you're doing or asking something. It's a bit like thinking out loud. The aim is to avoid questions standing out or seeming inappropriate. Here are some of the more common scenarios where you'll need to signpost:

Seemingly Unrelated Questions

If the patient comes in with nose pain and you start asking them about their knee, the patient will be confused. As a rule of thumb, if you ask the patient questions about body parts or systems that aren't directly physically connected to what they came in for, then signpost!

e.g.

"I know you have come in to discuss your acne – sometimes acne can be due to a hormonal problem – can I ask, how often do you get a period?"

Smoking and Drinking

A common pitfall is to not signpost these questions. You'll often want to get an understanding of a patient's general health and habits. If you've just finished talking about their bad back and you just come out with "Do you smoke or drink alcohol?" then the question is going to stand out. This damages your flow and damages your rapport. So tell the patient what you're doing:

e.g.

"Just to get an idea of your general health, could I ask, do you smoke? Do you drink?"

Signposting Common pitfall

Do NOT say that sometimes smoking or drinking might be contributing to their symptoms unless this is factually correct.

e.g.

“Sometimes back pain can be made worse by smoking”

This is untrue and makes you appear as if you do not know your medicine

Who do they live with?

In your mind, you probably want to know whether there's anyone that lives with them who can support them. But if you don't tell them you're thinking that, they may well think you're asking them whether they're single:

e.g. 1

"Do you live with a partner or anyone?"

e.g. 2

"It sounds like things are difficult at the moment. Do you have any support around you? Who is at home with you?"

"Does anyone else had home have a cough? By the way – who is at home?"

"Is there anyone at home to look after you?"

Physical Examinations

Always ask permission! For some people, any kind of physical exam or human contact can be stressful. People with autism or a history of abuse may well object to a physical exam, especially if asked improperly. But before/whilst you do that, explain what you're going to examine and why so they know what they're agreeing to. This is especially important before an intimate exam! As a general rule:

1. Explain why you think an examination would help
2. Tell them what you'll be examining (in minimal detail)
3. Ask their permission
4. Inform them that if the examination causes them any pain, to let you know and that you can stop
5. Offer a chaperone when required: This is obvious in the case of intimate examinations but may be appropriate in other cases (e.g. cardiovascular examination in a female where you have to ask the patient to remove their top).

e.g.

“To get an idea of what’s happening with your arm, I’d like to do a few physical examinations. Is that ok?”

A few notes on signposting:

- When doing a cardiovascular exam on a female, as the mitral valve is below the breast, explain clearly why you are placing your stethoscope there. Even if you're also female.
- Neurological exams may seem strange to a patient. It's a good idea to anticipate this, for example, by saying "some of these examinations might seem a bit strange but they'll give me a good idea of what's happening with your brain"

Talking About Sex

It's awkward and embarrassing. At least, it is for most people. And you can almost guarantee it will be for the patient! Knowing this, it's best to signpost these questions in advance.

One approach is to start by asking their permission for you to answer some '*personal questions*' so that they see that you empathise with their discomfort and know what to expect. You can then follow this up with the reason why you're asking. Not only will this reassure the patient that you have a good reason (and aren't just nosey) it will reassure the examiner that you know what you're doing. Alternatively, you could lead with your reason for asking.

Giving the patient **options** to choose from normalises sexual preferences and stops them feeling embarrassed.

Tone is also important for questions like this. When you're asking these questions, the patient needs to be reassured before they're willing to open up to you.

Sample phrases:

“Would it be ok, Mr X, for me to ask you a few personal questions? The symptoms you’re describing can sometimes point towards a sexually transmitted infection – could I ask, have you had any unprotected sexual intercourse recently?”

“Was this with a male partner or a female partner”

“Was this a regular partner or a casual partner?”

“Is it ok if I ask some more personal questions to ensure we aren’t missing an infection anywhere else?” “could I ask – did you have vaginal, oral or anal sex?”

“Sometimes this condition can affect people’s sex lives too Would it be ok for me to ask you some personal questions?”

“Sometimes a person’s sex life can contribute to their depression Would you mind if I asked you some personal questions?”

If you've gone down this route and the patient is still looking unsure or embarrassed (remember: picking up on visual cues is in the mark scheme too!) then a stock phrase to reassure them that you've "seen every illness imaginable"/ "seen most things" and that you're here "to treat and not to judge."

Practice Tip:

If you find saying words like "sex", "penis", "vagina" etc aloud difficult (don't be embarrassed if you do, you definitely aren't alone!) then practice saying them and other 'naughty words' out loud quickly and repeatedly in front of a mirror (and away from children!!) until you feel comfortable.

Clinical **Management**

These tips relate mostly to your clinical management

Structure

As a general rule, your structure should go something like this:

1. Ideas (and concerns) + Diagnosis
2. Positively framed prognosis
3. Expectations + Shared Management

This structure isn't meant to be rigid, especially when breaking bad news which has it's own section.

Example dialogue:

"I know you thought this was conjunctivitis, however from speaking to you and examining you I think there is inflammation affecting the deeper tissues of the eye [ideas + diagnosis]. This is treatable but left untreated it could be potentially sight threatening [positively framed prognosis] so it is important we start treatment right away. I know you were hoping for eye drops from me, today, however, what I would advise is that we get you seen in eye casualty risk away where they have specialist equipment to do more advanced checks [expectations + management]. Would you be happy to go to eye casualty today [shared]?"

Take Control!

In this part of the consult it should be you that's doing most of the talking!

The last thing you want is the patient taking over the conversation and interrogating you for answers. It looks unprofessional, will wreck your structure and will eat up time.

The following are some strategies you can use to prevent your management from turning into an interrogation:

1. Acknowledge or Address the ICE first

Patients will be more likely to interrupt if they feel they haven't been listened to or their needs aren't being met.

2. Pre-empt Questions and Concerns

If you can predict and answer any questions or concerns then you've prevented an interrogation before it's even happened.

e.g. 1

“You have a condition called heart-block. What I'd advise is...”

The term “heart block” can be very scary for a patient as most people associate heart conditions with heart attacks. A patient would probably start to look very worried and be interrupting you with questions about what it means.

“You have a condition called heart-block and although that may sound scary, it’s actually very treatable.”

Here, the patient’s concern has been dealt with in advance which prevents them from interrupting and allows you to continue with your management plan.

e.g. 2

You’ve just informed a patient that they will need to stop driving for 3 months but the patient relies on their car to get to work. They work shift work and not having a car means they wouldn’t be able to do their night shifts.

Solutions:

- Find out if there’s any friends or family that could help take them to work
- Offer them an amended duties note with suggestions for how they can use public transport

Dialogue:

Dr: “Unfortunately, with this condition you won’t be able to drive for three months. I imagine that’s

going to put you in a difficult position with work, am I correct?

Pt: "Yes, doctor"

Dr: "Do you know any colleagues who might be able to give you a lift? Some workplaces have ride share schemes too which might help you find someone?"

Pt: "No, there's nobody"

Dr: "Do you have any public transport nearby?"

Pt: "I do, but I work nights and it doesn't run then. Plus, I don't feel safe going out at night in my area"

Dr: "That's understandable. Would a Taxi be an option?"

Pt: "No. It's too expensive for me."

Dr: "Ok, well what I can do is give you an amended duties note. This will tell your employer that they need to adjust your hours until your eyes get better. I understand that may not be ideal for your manager but legally, they would have to make those adjustments. How does that sound?"

3. Negotiate in advance

If you know the patient is going to have a problem with the management of a condition, then use your negotiation strategy before they raise that problem!

This will save you time and really show that you've listened to the patient and have considered their psychosocial situation.

4. Avoid 'the pause'

When explaining a diagnosis or treatment or part of the management, people sometimes pause at the end of a sentence before fully explaining anything.

This opens you up for an interrogation!

Some people do it subconsciously, some are giving the patient room to talk and others might not be comfortable with talking at length for a long period of time.

Explaining conditions:

Your goal here is to explain something only once and avoid answering follow up questions. If the patient doesn't understand, they're going to ask and answering is going to take time which you don't have to spare. There are only two real tips for giving a good explanation:

1. Use one simple sentence
2. Avoid Jargon

A single simple sentence will save you time and increase the chances of the patient understanding because all they need to understand is one thing.

More information = More potential misunderstandings

Sometimes, an explanation doesn't even need to be scientifically very complex at all. It just needs to convey an idea.

By avoiding Jargon, you can avoid having to explain words and concept that the patient doesn't understand.

The only medical terminology you should be using in your explanation is the medical term you're explaining.

e.g. 1:

“Diabetes is a condition where not enough insulin is produced by your pancreas. This causes your blood sugar to go up”

Jargon words like ‘insulin’ and ‘pancreas’ could open you up to questions, meaning you’ll have to give even more explanations.

e.g. 2:

“Diabetes is a condition where the body becomes unable to process sugar”

The only medical terminology in this sentence is the condition you're explaining.

Side note:

Before you explain a diagnosis, it's often a good idea to ask what they know about the diagnosis if it's complex or particularly bad.

Practice tip:

Take a list of common conditions that you think you might come across. With your eyes closed, select one at random. Then, try and explain the condition in one sentence.

Delivering the Prognosis

Patients generally come to you when their life is being inconvenienced so it's important that you tell them how much longer that inconvenience will last and how it relates to them. This will show you empathise with them!

Not only can you score knowledge and rapport points, but you'll avoid any patient questions eating into your time too.

You'll probably be faced with two different scenarios which each require different approaches:

- Minor Ailments – positive outcome likely
- Serious Ailments – negative outcome likely

Minor Ailments:

Giving a prognosis for something minor can be as simple as saying “this is treatable” or “I should be able to give you something for that” and giving the patient a rough idea of how long they can expect it to last.

Side note:

It's always a good idea to relate the prognosis to the patient's ICE where possible. For example, if the patient has acne and wants it treated and cleared up in two days then it's a good idea to highlight now that it's not going to be possible. The patient might not be interested in the treatment then and you've saved yourself from going through all the options.

Sharing Management

Ticking the 'shared management' box is all about how you phrase the treatment option(s).

A common error is to say something like this:

***"I'm going to** prescribe you some paracetamol and that should help ease the pain"*

In this example, the patient didn't get a choice. This was the all-knowing doctor who's commands are law!

A mean actor might kick up a fuss about how they hate paracetamol and don't want it because they only use herbal medicine. This will cost you time and rapport.

An even sneakier actor might just agree to your treatment after conveniently forgetting that they're allergic to paracetamol. Then you've just killed your patient and the GMC letter is in the post!

The general principle behind good shared management is that you give the patient the option(s) and let them choose. Even if there's only one option.

One treatment option

If there's only one option, you can start off with *"The treatment for (condition) is..."* and finish with *"would you like me to (do the treatment)"*.

Multiple Treatment Options:

At this point in the consultation, you probably know what the best treatment would be and what the alternatives are. To share the management effectively (and score some knowledge points with the examiner), you've got to give the patient those options!

But it's not enough to just give a list of options and let them pick any of them!

Let's say a patient has a choice of two drugs to choose from. If you don't explain the pros and cons of the drugs or which one is better and why, the patient is going to interrogate you for the answer. And unfortunately for you, patients don't have a timer to worry about!

This is where you become a salesperson and the optimum treatment is the hot new product!

Lead with your preferred treatment and give the pros followed by the cons. Then go through the other options, relating the pros and cons to your preferred treatment and why they aren't as good.

e.g. 1

“For your elbow you could try a steroid injection or physiotherapy”

e.g. 2

“Physiotherapy would really help your elbow because it would strengthen the muscles around the joint, this would prevent the issue re-occurring, however it won’t give you immediate results. The other option is a steroid injection. If this works it will take effect very quickly, however it doesn’t work for everyone and we don’t recommend this as a first choice because when we inject the joint we break the skin so there is a chance of introducing infection and there is a risk of damaging nerves in the area. Would you be happy to try physiotherapy first?”

Breaking Bad News

Get this wrong and the patient will either be blindsided and a mess for the rest of the consultation, or they might leave thinking everything is fine.

That's what you want to avoid!

The most common error is to just tell the patient without prior warning, which leads to them blindsided. You've got to slowly prepare them for the news and then reassure them once you've given it. Try taking the following approach:

1. Explain your thought process (Warning shot 1)
2. Best case scenario
3. Warning shot 2
4. Worst case scenario
5. Acknowledge how they feel
6. What happens next
7. Follow up

Example dialogue:

“Bleeding from down below is something we take seriously, particularly in women of your age [warning shot 1]”

“There may be a simple explanation for this [best case scenario] but in a small minority of cases the bleeding can be due to something more serious [warning shot 2] like a cancer of the womb.”

“I appreciate that this isn’t easy to hear [acknowledge how they feel]...”

“Could I explain what I recommend we do next?”

“What I would advise is that we refer you to see a specialist within two weeks. They would most likely have a look inside your womb with a camera and take a small sample of womb tissue. [what happens next] This would give us some answers. Are you happy for me to make this referral?”

“Shall we set another appointment for after you’ve seen the specialist? [follow up]”

Side note:

Breaking bad news is not limited to what **WE** would classify as serious diagnoses (e.g. cancer, multiple sclerosis). Other things can feel like very bad news to a patient.

Examples include

- A possible ectopic pregnancy
- Possible PCOS
- Diabetes (A good tip, when giving the diagnosis is to ask “does this come as a surprise to you?” to gauge how much of a shock this is)
- Telling a patient they are pregnant
- Telling a patient in a long term monogamous relationship that they have an STI
- Heart failure (Remember if you say “heart failure” to a lay person, it is a scary thought to think their heart is failing - so it’s important to explain this is a medical term for when the heart is not pumping as efficiently as it could but there are a lot of things that can be done to help)

Negotiating

This is another topic where literally *thousands* of books have been written on it. The guidance here is what I've found to be effective for CSA candidates and in my practice too.

A good strategy is to:

- 1. Acknowledge their emotion*
- 2. Empathise with their position*
- 3. Deal with blockers (if possible)*
- 4. Sell your solution to them.*

1. Acknowledging their emotion:

This shows that you've noticed the patients concern and that they've been heard. This will take their emotional level down a bit and reduce the chances of entering a shouting match with them. It only has to be a small sentence:

"I understand that going to hospital can be scary"

"I can appreciate you can't see the point in taking the pills"

"I know it would be a difficult change to make"

2. Empathising with their position:

This shows that you've understood them and have listened and can appreciate their perspective. If the patient thinks you understand their point of view, they'll trust that you'd do the same in their position and are more likely to accept your option. Try and put in something which they relate to here too but avoid giving personal anecdotes.

“...especially after your last hospital visit”

“and it must be difficult to find the motivation when you aren't experiencing any symptoms”

“...and big changes often create a lot of stress”

3. Dealing with blockers:

How you deal with this will depend on the situation. However, after you've explained the options (remember your shared management "you could...") it's often a good idea to acknowledge that the solution isn't perfect before selling your solution.

"What I could do is inform the ambulance and hospital staff about your experiences and they'll do what they can to make everything comfortable. I appreciate it isn't ideal..."

"If you'd like, I can get you a box which will help you keep track of when to take your pills and which pills you've taken. I understand, that it's an extra inconvenience..."

"But I could always book you in for a follow up appointment and we can talk about the stress and ways to manage it. I appreciate that it's not ideal and it won't take away the stress but..."

4. Selling your solution:

Now that you've shown the patient that you're on their side, gained their trust and given them a work around for their blockers, now is the time to sell your solution! And the best way to do that is to show them why their idea is bad for them and why your idea is a LOT better.

“If you don't go into hospital today then there is a chance that this could become potentially life threatening, but with appropriate treatment in hospital, I expect you will feel a lot better”

“But although you aren't experiencing any side effects now, not taking the pills is going to create very big problems for you later. By taking them, you'll be able to skateboard and skydive for much much longer”

“...not changing your diet would seriously impact your health a few years down the line whereas these changes would keep you happy, healthy and enjoying life for a lot longer.”

Putting the examples together:

“I understand that going to hospital can be scary, especially after your last hospital visit. What I could do is inform the ambulance and hospital staff about your experiences and they’ll do what they can to make everything comfortable. If you don’t go into hospital today then there is a chance that this could become potentially life threatening, but with appropriate treatment in hospital, I expect you will feel a lot better

“I can appreciate you can’t see the point in taking the pills and it must be difficult to find the motivation when you aren’t experiencing any symptoms. If you’d like, I can get you a box which will help you keep track of when to take your pills and which pills you’ve taken. I understand, that it’s still an extra inconvenience, but although you aren’t experiencing any side effects now, not taking the pills is going to create very big problems for you later. By taking them, you’ll be able to skateboard and skydive for much much longer”

“I know it would be a difficult change to make and big changes often create a lot of stress. But I could always book you in for a follow up appointment and we can talk about the stress and ways to manage it. I appreciate that it’s not ideal and it won’t take away the stress but not making these changes would seriously impact your health a few years down the line whereas these changes would keep you happy, healthy and enjoying life for a lot longer.”

Appendix

This is information that I hope you'll find useful but hasn't been given any sections or formatting yet.

Medlighten: Communication Examples

ADMISSION CASES

How would you deal with a patient refusing admission?

Admission cases often come with blockers:

- Unwell relative: Call emergency social care or ICT as appropriate
- Pets: Call RSCPA
- Patient has had a bad experience in hospital: Explain this to hospital/ambulance staff

e.g.

John (61) had central crushing chest pain spreading to his left arm 20 minutes ago with sweating and shortness of breath whilst he was sitting reading a book. Your practice does not have an ECG. You explain to John he needs to go to the hospital because you are worried about a possible heart attack. John explains this will not

be possible because he is self employed running a shop and cannot afford to lose a day's income. There is no one else available to look after the shop.

What do you need to do?

Empathise with John to build a connection:

“I can imagine it must be really difficult to consider taking time off work - particularly because you are self employed”

Explain ultimate risk and check his understanding:

“however, the reason I would strongly advise that we get you seen in hospital today is because if this chest pain is coming from the heart then it could be potentially life threatening - does this make sense?”

Find out why finances are an issue - is this something you can address?

If John is adamant he doesn't want to go to hospital, despite your best efforts at attempting to persuade him - use negotiation skills.

What is John's main issue? Potential loss of money from the business

How will his plan worsen the issue? If admission is delayed he might be in hospital for longer

“I know finances are an issue for you. However, as well as being potentially life threatening, if you do not go to hospital today, there is a risk that if things were to get worse and you were admitted later that you may be in hospital longer; are you with me?”

As a last resort, check his capacity:

“John, I want to be absolutely sure you have understood what we have discussed. Is it possible for you to repeat in your own words what the risks would be if you didn't go to hospital right away?”

Always leave the door open

Offer to refer to rapid access chest pain clinic

If John changes his mind at any time, explain he can contact you

NON COMPLIANT PATIENTS

Darren, 28 is a Type 1 diabetic at University. He has had multiple admissions for DKA but he thinks it's fine because he feels fine once he is out of hospital. He thinks diabetes only becomes an issue as you get older. He is busy enjoying nights out and is determined to live life to the full and fit in with his friends. He finds insulin inconvenient to carry around.

What do you say to Darren?

“So it sounds like you are making the most of university life.”

Do you know why we are so keen for patients to closely monitor their blood sugars?”

“Would you like me to explain the reasons why?”

“Can you see why we are so keen for you to control your diabetes?”

“I can see how inconvenient this is, given your busy lifestyle [Acknowledging his perspective] - shall we discuss how we could make things easier?” (Insulin pump)

Compliance can be an issue for many reasons

1. The patient has a busy schedule
2. The patient does not appreciate the risks of their chronic illness being left untreated
3. The patient does not understand why the medications are needed
4. There is a stigma in seeking help
5. The patient keeps forgetting their medication

NEGOTIATION 1: NOT GIVING THE PATIENT WHAT THEY WANT

You need to

1. Find out the patient's underlying wishes
2. Acknowledge their perspective
3. Then:
 - Either
 - Explain their idea isn't good
 - Explain why your idea is better and will still get them what they want
 - OR
 - Explain why your idea will achieve the same thing in a different way

e.g. 1

An acutely anxious patient demanding diazepam

a. What is the patient's underlying wish: To feel relaxed

b. What do you do?

i. Acknowledge their perspective:

"I appreciate that you are feeling very anxious. You are right that diazepam can relieve anxiety quickly"

ii. Explain why their idea isn't good:

"However, the difficulty with diazepam is that it can become addictive and lose its effectiveness with time"

iii. Explain why your idea is better

"There is another medication called propranolol which can also help you calm down quickly and doesn't have these issues. Would you like to give this a try?"

e.g. 2

A patient with depression asking for a mitigation letter for university but asking you not to write “depression” in the letter because they are worried this will affect their grades:

a. What is the patient’s underlying wish: To do well with their degree

b. Acknowledge their perspective:

“I appreciate you don’t want people knowing about depression, that’s understandable.”

c. Explain why your idea is better than theirs:

“However depression is quite common and I suspect the university have seen a lot of cases of depression before and will be trained to support you. If they know about your depression it would help them give you the appropriate support to help you get the results you deserve.”

e.g. 3

A patient with acne, who works in a call centre requesting a 3 month sick note due to social anxiety

a. What is the patient's underlying wish: To avoid feeling self-conscious

b. What do you do:

i. Acknowledge their perspective:

"I can see how much the acne is affecting your confidence. In the short term, if you wanted a sick note I think this would be reasonable"

ii. Explain why your idea will achieve the same thing (feeling better about herself) in a different way:

"However in the long term, shall we discuss ways we can help you feel better about yourself so that you feel more able to return to work?"

NEGOTIATION: TAKING SOMETHING AWAY

1. Explain your reasoning
2. Acknowledge the impact on their life
3. Give a plan B

Stopping medication when the patient has a medication over-overuse headache:

Explain your reasoning:

“The type of headache you currently have sounds like a medication-overuse headache. [Acknowledge it doesn’t make sense] I know it seems counterintuitive but sometimes medication used to treat headaches can actually worsen headaches, particularly co-codamol. [Manage expectations] In terms of treatment - this can be a tricky condition to treat because the evidence would suggest it is best stop all your medication and allow sufficient time for it to leave your system. Sometimes things can get worse before they get better.”

“What I would advise we do is stop your paracetamol and ibuprofen right away and gradually reduce your codeine. It would normally take 1 - 2 months for the headache to settle.”

Acknowledge the impact on their life:

“I know this might make things difficult.”

Give a Plan B:

“Sometimes people prefer to take a bit of time off work during the first 2 – 3 weeks; if it would help, I can offer you a sick note [when you take something away (meds), give something in return (med3)]”

Explaining to a patient with epilepsy that they can't drive

Explain your reasoning:

“When someone has had a fit, we would advise them not to drive for at least 6 months because if they were to have a fit whilst driving it could put them or other road users at risk.”

Acknowledge the impact on their life:

“I appreciate this may make things difficult for you because you are very reliant on your car.”

Give a plan B:

“Shall we discuss ways around this?”

REASSURING PATIENTS

Sophie (29) has tension headaches but is worried she has a brain tumour:

“From speaking to you and examining you, this doesn’t look like a brain tumour because you don’t have any worrying symptoms, for example your vision is normal and your examination was completely normal.”

“I suspect that to start with you had something called a tension headache which is a headache brought on when we subconsciously tense up our muscles due to stress. Does this make sense? [asking if this makes sense is a good way of assessing whether Sophie believes you]”

“There are a things which can help relax those tensed up muscles.”

“Would you like me to go through a few techniques that might help? [SHARED]”

If a patient doesn’t believe you for example in the case of medically unexplained symptoms, IBS or tension headaches – a good question to ask is:

“I don’t know if this is the case, but have you noticed your symptoms get worse when you are stressed?”

PATIENTS LACKING INSIGHT

Sarah, 27 has been advised to see you by her fiancé. She is spending 7 hours a day cleaning and washes her hands 10 times a day with bleach. Sarah does not think there is anything wrong with this and is adamant she is “not mad”.

How do you get Sarah to see she has a problem?

Data gathering: Find out the negative effects on her life INDIRECTLY

“So you are spending 7 hours a day cleaning and working full time as well. That sounds very busy. How do you manage to fit everything in?”

Hopefully Sarah will admit this is a struggle

Clinical Management: Remind Sarah of the negative aspects of cleaning:

“It sounds like whilst cleaning is really important to you, it is causing you some problems.”

Follow up with a neutral statement:

“Shall we discuss ways we can make things easier for you?”